

Continent Cutaneous Diversion (Indiana Pouch)

WARWIKI

Patient Information · An inside pouch you empty with a catheter, with no bag

A continent cutaneous diversion is a pouch made from your own bowel. It stores urine inside your body. A small opening on your belly (a stoma) stays closed between uses. You do not wear a bag. Instead, you empty the pouch several times a day by passing a thin tube (a catheter) through the opening. The Indiana pouch is one well-known type.

About This Procedure

A piece of **large and small bowel** is reshaped into a soft **pouch** inside the belly. The ureters (the tubes from the kidneys) drain into it. A narrow channel with a **one-way valve** is brought to the skin as a small, flat stoma. It is sometimes placed in the belly button.

- You pass a catheter to drain the pouch on a **schedule** your team sets.
- You **rinse** the pouch to clear mucus. The bowel keeps making mucus for life.
- **No bag.** A small pad or bandage covers the stoma.

It suits people who want no bag, cannot have or do not want a neobladder, and can **catheterize for life**. That takes steady hands and good eyesight, or a helper.

Benefits and Risks in Numbers

about
9 in 10

stay dry between
catheterizations

about
15 in 100

get a narrowed
stoma that can
make the catheter
hard to pass

about
1 in 2

need another
procedure over the
years; many are
small

Numbers are averages from published studies. Your own risk may differ; ask your team.

Learn the Terms

Continent diversion

A urine system that does not leak. You choose when it empties.

Indiana pouch

A continent pouch built from the right side of the large bowel and a bit of small bowel.

Pouch

The inside sac, made from bowel, that holds urine.

Stoma

The small skin opening you pass the catheter through.

Catheterize

To pass a thin tube through the stoma to drain the pouch.

Irrigate

To rinse the pouch with salt water to clear mucus.

Ureters

The tubes that carry urine from the kidneys to the pouch.

Mucus

Slippery fluid the bowel makes. Clearing it is part of daily care.

Will it hurt? You are asleep for the surgery, so you feel nothing. Afterward, your belly will be sore, and you get pain medicine. The bowel takes a few days to start working again. You wake up with a tube draining the pouch and thin stents in the ureters.

In the hospital

Walk early. Eat again slowly. Learn to rinse the pouch tube.

First weeks at home

The tube stays until the pouch heals. You may give yourself shots to prevent clots for about 4 weeks.

Tube out

Start catheterizing, often at first, then less often as your team advises.

For life

Catheterize, rinse, and keep your check-ups.

How to Get Ready

- Follow your instructions about eating, drinking and which medicines to stop. Your team will tell you if you need a bowel prep.
- Your team plans the **stoma spot** with you and makes sure you, or a helper, can **catheterize for life**.
- Stop smoking**, and arrange help at home.

Tell your team ahead of time if you:

- Take a blood thinner, or have kidney or liver problems
- Have bowel disease, or trouble with your hands or eyesight

What Happens During Surgery

- You are asleep and get antibiotics.
- A piece of bowel is set aside. The rest of the bowel is joined back together.
- The bowel is shaped into a pouch. A narrow channel with a valve is brought to the skin.
- The ureters are joined to the pouch. A tube and stents stay in while it heals.

Your schedule: many people end up emptying about every 4–6 hours in the day and once at night. Yours depends on how much the pouch holds. **For life:** blood tests (kidney function, acid level, vitamin B12) and kidney scans.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- A catheter that is getting harder to pass, or new leaking from the stoma
- Redness, swelling or fluid leaking from the cut

Go to an emergency department if:

- You cannot pass the catheter, or no urine comes out. A full pouch must be drained soon.
- Bad belly pain, vomiting, or no gas or stool
- Chest pain, trouble breathing, or heavy bleeding
- You cannot reach the office and are worried

Three Things to Remember

- The pouch stores urine inside you. You empty it with a catheter, with no bag.
- Never skip or stretch out your emptying schedule. Rinse out mucus as shown.
- Keep your check-ups for life, even when you feel well.